pharmaniaga

ZINOX

Linezolid 600 mg

DESCRIPTION
White to off white, oblong-shaped film coated tablet with Pharmaniaga logo on one side and plain on the other side.

COMPOSITION
Each film-coated tablet contains Linezolid 600 mg.

PHARMACOLOGICAL PROPERTIES
PHARMACODYNAMICS
Linezolid is a synthetic antibacterial agent that belongs to a new class of antimicrobials, the oxazolidinones. It has *in vitro* activity against aerobic Gram-positive bacteria, some Gram-negative bacteria and anaerobic microorganisms. Linezolid selectively inhibits bacterial protein synthesis via a unique mechanism of action. Specifically, it binds to a site on the bacterial ribosome (23S of the 50S subunit) and prevents the formation of a functional 70S initiation complex which is an essential component of the translation process.

Susceptibility: Only microorganisms relevant to the given clinical indications are presented here.

Category
Susceptible organisms
Gram positive aerobes:
Enterococcus faecalis
*Enterococcus faecium**
*Staphylococcus aureus**
Coagulase negative *staphylococci*
*Streptococcus agalactiae**
*Streptococcus pneumoniae**
*Streptococcus pyogenes**
Group C *streptococci*
Group G *streptococci*
Gram positive anaerobes:
Clostridium perfringens
Peptostreptococcus anaerobius
Peptostreptococcus species
Resistant organisms
Haemophilus influenzae
Moraxella catarrhalis
Neisseria species
Enterobacteriaceae
Pseudomonas species

* Clinical efficacy has been demonstrated for susceptible isolates in approved clinical indications.

Resistance
Cross resistance: Linezolid's mechanism of action differs from those of other antibiotic classes. Based on reported data from clinical isolates (including methicillin-resistant *staphylococci*, vancomycin-resistant *enterococci*, and penicillin- and erythromycin-resistant *streptococci*) indicate that linezolid is usually active against organisms which are resistant to one or more other classes of antimicrobial agents.
Resistance to linezolid is associated with point mutations in the 23S rRNA.
As documented with other antibiotics when used in patients with difficult to treat infections and/or for prolonged periods, emergent decreases in susceptibility have been observed with linezolid. Resistance to linezolid has been reported in *enterococci, Staphylococcus aureus* and coagulase negative *staphylococci*. This generally has been associated with prolonged courses of therapy and the presence of prosthetic materials or undrained abscesses. When antibiotic-resistant organisms are encountered in the hospital it is important to emphasize infection control policies.

PHARMACOKINETICS
Absorption: Linezolid is rapidly and extensively absorbed after oral dosing. Maximum plasma concentrations are reached within 2 hours of dosing. Absolute oral bioavailability of linezolid is complete (approximately 100%). Absorption is not significantly affected by food.

Distribution: Volume of distribution at steady-state averages at about 40-50 litres in healthy adults and approximates to total body water. Plasma protein binding is about 31% and is not concentration dependent.

Based on reported data, the pharmacokinetic information generated in pediatric patients with ventriculoperitoneal shunts showed variable cerebrospinal fluid (CSF) linezolid concentrations following single and multiple dosing of linezolid; therapeutic concentrations were not consistently achieved or maintained in the CSF. Therefore, the use of linezolid for the empiric treatment of paediatric patients with central nervous system infections is not recommended.

Metabolism: Linezolid is primarily metabolized by oxidation of the morpholine ring resulting mainly in the formation of two inactive open-ring carboxylic acid derivatives: the aminoethoxyacetic acid metabolite and the hydroxyethyl glycine metabolite. The hydroxyethyl glycine metabolite is the predominant human metabolite and is believed to be formed by a non-enzymatic process. The aminoethoxyacetic acid metabolite is less abundant. Other minor, inactive metabolites have been characterised.

Elimination: In patients with normal renal function or mild to moderate renal insufficiency, linezolid is primarily excreted under steady-state conditions in the urine as hydroxyethyl glycine metabolite (40%), parent drug (30%) and aminoethoxyacetic acid metabolite (10%). Virtually no parent drug is found in the feces while approximately 6% and 3% of each dose appears as hydroxyethyl glycine metabolite and aminoethoxyacetic acid metabolite respectively. The elimination half-life of linezolid averages at about 5-7 hours.

Non-renal clearance accounts for approximately 65% of the total clearance of linezolid. A small degree of non-linearity in clearance is observed with increasing doses of linezolid. This appears to be due to lower renal and non-renal clearance at higher linezolid concentrations. However, the difference in clearance is small and is not reflected in the apparent elimination half-life.

Special Populations
Patients with renal insufficiency: In patients with severe renal insufficiency (i.e. creatinine clearance <30 ml/min), there was a 7-8-fold increase in exposure to the two primary metabolites of linezolid in the plasma after single doses of 600 mg. However, there was no increase in AUC of parent drug. Although there is some removal of the major metabolites of linezolid by hemodialysis, metabolite plasma levels after single 600 mg doses were still considerably higher following dialysis than those observed in patients with normal renal function or mild to moderate renal insufficiency.
Patients with hepatic insufficiency: Limited data indicate that the pharmacokinetics of linezolid, aminoethoxyacetic acid metabolite and hydroxyethyl glycine metabolite are not altered in patients with mild to moderate hepatic insufficiency (i.e. Child-Pugh class A or B). The pharmacokinetics of linezolid in patients with severe hepatic insufficiency (i.e. Child-Pugh class C) have not been evaluated. However, as linezolid is metabolized by a non-enzymatic process, impairment of hepatic function would not be expected to significantly alter its metabolism.

Children and adolescents (<18 years old): In adolescents (12 to 17 years old), linezolid pharmacokinetics were similar to that in adults following a 600 mg dose. Therefore, adolescents administered 600 mg every 12 hours daily will have similar exposure to that observed in adults receiving the same dosage.

In children 1 week to 12 years old, administration of 10 mg/kg every 8 hours daily gave exposure approximating to that achieved with 600 mg twice daily in adults.

In neonates up to 1 week of age, the systemic clearance of linezolid (based on kg body weight) increases rapidly in the first week of life. Therefore, neonates given 10 mg/kg every 8 hours daily will have the greatest systemic exposure on the first day after delivery. However, excessive accumulation is not expected with this dosage regimen during the first week of life as clearance increases rapidly over that period.

Elderly patients: The pharmacokinetics of linezolid are not significantly altered in elderly patients (65 years or older).

Female patients: Females have a slightly lower volume of distribution than males and the mean clearance is reduced by approximately 20% when corrected for body weight. Plasma concentrations are higher in females and this can partly be attributed to body weight differences. However, because the mean half-life of linezolid is not significantly different in males and females, plasma concentrations in females are not expected to substantially rise above those known to be well tolerated and, therefore, dose adjustment are not required.

INDICATIONS
Therapeutic indications: Linezolid formulations are indicated in the treatment of the following infections caused by susceptible strains of the designated microorganisms. Linezolid is active against Gram-positive bacteria only. Linezolid has no clinical activity against Gram-negative pathogens. Specific Gram-negative therapy is required if a concomitant Gram-negative pathogen is documented or suspected.

Vancomycin-Resistant *Enterococcus faecium* infections, including cases with concurrent bacteremia.

Nosocomial pneumonia caused by *Staphylococcus aureus* (methicillin-susceptible and - resistant strains), or *Streptococcus pneumoniae* (penicillin-susceptible strains). Combination therapy may be clinically indicated if the documented or presumptive pathogens include Gram-negative organisms.

Complicated skin and skin structure infections including diabetic foot infections, without concomitant osteomyelitis caused by *Staphylococcus aureus* (methicillin-susceptible and -resistant strains), *Streptococcus pyogenes*, or *Streptococcus agalactiae*. Linezolid has not been studied in the treatment of decubitus ulcers. Combination therapy may be clinically indicated if the documented or presumptive pathogens include Gram negative organisms.

Uncomplicated skin and skin structure infections caused by *Staphylococcus aureus* (methicillin-susceptible only) or *Streptococcus pyogenes*.

Community-acquired pneumonia caused by *Streptococcus pneumoniae* (penicillin-susceptible strains only), including cases with concurrent bacteremia, or *Staphylococcus aureus* (methicillin-susceptible strains only).

Due to concerns about inappropriate use of antibiotics leading to an increase in resistant organisms, prescribers should carefully consider alternatives before initiating treatment with linezolid in the outpatient setting.

Appropriate specimens for bacteriological examination should be obtained in order to isolate and identify the causative organisms and to determine their susceptibility to linezolid. Therapy may be instituted empirically while awaiting the results of these tests. Once these results become available, antimicrobial therapy should be adjusted accordingly.

RECOMMENDED DOSAGE
Patients whose therapy is started with Linezolid injection may be switched to Linezolid tablets, with no dosage adjustment.

Dosage Guidelines for Zinox Tablet 600 mg

Infection*	Dosage and Route of Administration: Pediatric Patients† (Birth through 11 Years of Age)	Adults and Adolescents (12 Years and Older)	Recommended Duration of Treatment (consecutive days)
Complicated skin and skin structure infections Community-acquired pneumonia, including concurrent bacteremia Nosocomial pneumonia	10 mg/kg IV or oral‡ q8h	600mg IV or oral‡ q12h	10 to14
Vancomycin-resistant *Enterococcus faecium* infections, including concurrent bacteremia	10 mg/kg IV or oral‡ q8h	600mg IV or oral‡ q12h	14 to 28
Uncomplicated skin and skin structure infections	<5 yrs: 10mg/kg oral‡ q8h 5-11 yrs: 10mg/kg oral‡ q12h	Adults: 400mg oral‡ q12h Adolescents : 600mg oral‡ q12h	10 to14

* Due to the designated pathogens

† Neonates <7 days: Most pre-term neonates <7 days of age (gestational age <34 weeks) have lower systemic linezolid clearance values and larger AUC values than many full-term neonates and older infants. These neonates should be initiated with a dosing regimen of 10 mg/kg q12h. Consideration may be given to the use of 10 mg/kg q8h regimen in neonates with a sub-optimal clinical response. All neonatal patients should receive 10 mg/kg q8h by 7 days of life.

‡ Oral dosing using Linezolid Tablets.

Elderly patients: No dose adjustment is required.

Patients with renal insufficiency: No dose adjustment is required.
Patients with severe renal insufficiency (i.e., CL_{CR} < 30 ml/min): No dose adjustment is required. Due to the unknown clinical significance of higher exposure (up to 10-fold) to the two primary metabolites of linezolid in patients with severe renal insufficiency, linezolid should be used with special caution in these patients and only when the anticipated benefit is considered to outweigh the theoretical risk.

As approximately 30% of a linezolid dose is removed during 3 hours of hemodialysis, linezolid should be given after dialysis in patients receiving such treatment. The primary metabolites of linezolid are removed to some extent by hemodialysis, but the concentrations of these metabolites are still very considerably higher following dialysis than those observed in patients with normal renal function or mild to moderate renal insufficiency.

Therefore, linezolid should be used with special caution in patients with severe renal insufficiency who are undergoing dialysis and only when the anticipated benefit is considered to outweigh the theoretical risk.
To date, there is no experience of linezolid administration to patients undergoing continuous ambulatory peritoneal dialysis (CAPD) or alternative treatments for renal failure (other than haemodialysis).

Patients with hepatic insufficiency: No dose adjustment is required. It is recommended that linezolid should be used in such patients only when the anticipated benefit is considered to outweigh the theoretical risk.

ROUTE OF ADMINISTRATION
Oral.

CONTRAINDICATIONS
Linezolid is contraindicated in patients who have previously demonstrated hypersensitivity to linezolid or any of the other product components.

Linezolid should not be used in patients taking any medicinal product which inhibits monoamine oxidases A or B (e.g. phenelzine, isocarboxazid, selegiline, moclobemide) or within two weeks of taking any such medicinal product.

Unless there are facilities available for close observation and monitoring of blood pressure, linezolid should not be administered to patients with the following underlying clinical conditions or on the following types of concomitant medications:

- Patients with uncontrolled hypertension, phaeochromocytoma, carcinoid, thyrotoxicosis, bipolar depression, schizoaffective disorder, acute confusional states.

- Patients taking any of the following medications: serotonin re-uptake inhibitors, tricyclic antidepressants, serotonin 5-HT_1 receptor agonists (triptans), directly and indirectly acting sympathomimetic agents (including the adrenergic bronchodilators, pseudoephedrine and phenylpropanolamine), vasopressive agents (e.g. epinephrine, norepinephrine), dopaminergic agents (e.g. dopamine, dobutamine), pethidine or buspirone.

Linezolid and its metabolites may pass into breast milk and accordingly, breast-feeding should be discontinued prior to and throughout administration.

WARNINGS AND PRECAUTIONS
Myelosuppression: Myelosuppression (including anaemia, leucopenia, pancytopenia and thrombocytopenia) has been reported in patients receiving linezolid. In cases where the outcome is known, when linezolid was discontinued, the affected haematologic parameters have risen toward pre-treatment levels. The risk of these effects appears to be related to the duration of treatment. Elderly patients treated with linezolid may be at greater risk of experiencing blood dyscrasias than younger patients. Thrombocytopenia may occur more commonly in patients with severe renal insufficiency, whether or not on dialysis. Therefore, close monitoring of blood counts is recommended in patients who: have pre-existing anaemia, granulocytopenia or thrombocytopenia; are receiving concomitant medications that may decrease haemoglobin levels, depress blood counts or adversely affect platelet count or function; have severe renal insufficiency; receive more than 10-14 days of therapy. Linezolid should be administered to such patients only when close monitoring of haemoglobin levels, blood counts and platelet counts is possible.

If significant myelosuppression occurs during linezolid therapy, treatment should be stopped unless it is considered absolutely necessary to continue therapy, in which case intensive monitoring of blood counts and appropriate management strategies should be implemented.
In addition, it is recommended that complete blood counts (including haemoglobin levels, platelets, and total and differentiated leucocyte counts) should be monitored weekly in patients who receive linezolid regardless of baseline blood count.

A higher incidence of serious anaemia was reported in patients receiving linezolid for more than the maximum recommended duration of 28 days. These patients more often required blood transfusion.

Cases of anaemia requiring blood transfusion have also been reported post marketing, with more cases occurring in patients who received linezolid therapy for more than 28 days.

Cases of sideroblastic anaemia have been reported post-marketing. Where time of onset was known, most patients had received linezolid therapy for more than 28 days. Most patients fully or partially recovered following discontinuation of linezolid with or without treatment for their anaemia.

212mm x 277mm Font Size: 6.5pt

Antibiotic-associated diarrhoea and colitis: Antibiotic-associated diarrhoea and antibiotic-associated colitis, including pseudomembranous colitis and *Clostridium difficile*-associated diarrhoea, has been reported in association with the use of nearly all antibiotics including linezolid and may range in severity from mild diarrhoea to fatal colitis. Therefore, it is important to consider this diagnosis in patients who develop serious diarrhoea during or after the use of linezolid. If antibiotic-associated diarrhoea or antibiotic-associated colitis is suspected or confirmed, ongoing treatment with antibacterial agents, including linezolid, should be discontinued and adequate therapeutic measures should be initiated immediately. Drugs inhibiting peristalsis are contraindicated in this situation.

Lactic acidosis: Lactic acidosis has been reported with the use of linezolid. Patients who develop signs and symptoms of metabolic acidosis including recurrent nausea or vomiting, abdominal pain, a low bicarbonate level, or hyperventilation while receiving linezolid should receive immediate medical attention. If lactic acidosis occurs, the benefits of continued use of linezolid should be weighed against the potential risks.

Mitochondrial dysfunction: Linezolid inhibits mitochondrial protein synthesis. Adverse events, such as lactic acidosis, anaemia and neuropathy (optic and peripheral), may occur as a result of this inhibition; these events are more common when the drug is used longer than 28 days.

Serotonin syndrome: Spontaneous reports of serotonin syndrome associated with the co-administration of linezolid and serotonergic agents, including antidepressants such as selective serotonin reuptake inhibitors (SSRIs) have been reported. Co-administration of linezolid and serotonergic agents is therefore contraindicated except where administration of linezolid and concomitant serotonergic agents is essential. In those cases, patients should be closely observed for signs and symptoms of serotonin syndrome such as cognitive dysfunction, hyperpyrexia, hyperreflexia and incoordination. If signs or symptoms occur physicians should consider discontinuing either one or both agents; if the concomitant serotonergic agent is withdrawn, discontinuation symptoms can occur.

Peripheral and optic neuropathy: Peripheral neuropathy, as well as optic neuropathy and optic neuritis sometimes progressing to loss of vision, have been reported in patients treated with linezolid; these reports have primarily been in patients treated for longer than the maximum recommended duration of 28 days.
All patients should be advised to report symptoms of visual impairment, such as changes in visual acuity, changes in colour vision, blurred vision, or visual field defect. In such cases, prompt evaluation is recommended with referral to an ophthalmologist as necessary. If any patients are taking linezolid for longer than the recommended 28 days, their visual function should be regularly monitored.

If peripheral or optic neuropathy occurs, the continued use of linezolid should be weighed against the potential risks.

There may be an increased risk of neuropathies when linezolid is used in patients currently taking or who have recently taken antimycobacterial medications for the treatment of tuberculosis.

Convulsions: Convulsions have been reported to occur in patients when treated with Linezolid. In most of these cases, a history of seizures or risk factors for seizures was reported. Patients should be advised to inform their physician if they have a history of seizures.

Monoamine oxidase inhibitors
Linezolid is a reversible, non-selective inhibitor of monoamine oxidase (MAOI); however, at the doses used for antibacterial therapy, it does not exert an anti-depressive effect. There are very limited data from drug interaction and on the safety of linezolid when administered to patients with underlying conditions and/or on concomitant medications which might put them at risk from MAO inhibition. Therefore, linezolid is not recommended for use in these circumstances unless close observation and monitoring of the recipient is possible.

Use with tyramine-rich foods: Patients should be advised against consuming large amounts of tyramine-rich foods.

Superinfection: The effects of linezolid therapy on normal flora have not been evaluated in clinical trials.

The use of antibiotics may occasionally result in an overgrowth of non-susceptible organisms. For example, approximately 3% of patients receiving the recommended linezolid doses experienced drug-related candidiasis. Should superinfection occur during therapy, appropriate measures should be taken.

Special populations: Linezolid should be used with special caution in patients with severe renal insufficiency and only when the anticipated benefit is considered to outweigh the theoretical risk.

It is recommended that linezolid should be given to patients with severe hepatic insufficiency only when the perceived benefit outweighs the theoretical risk.

Impairment of fertility
Linezolid reversibly decreased fertility and induced abnormal sperm morphology in adult male rats at exposure levels approximately equal to those expected in humans; possible effects of linezolid on the human male reproductive system are not known.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

Patients should be warned about the potential for dizziness or symptoms of visual impairment whilst receiving linezolid and should be advised not to drive or operate machinery if any of these symptoms occurs.

INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

Monoamine oxidase inhibitors: Linezolid is a reversible, non-selective inhibitor of monoamine oxidase (MAOI). There are very limited data from drug interaction and on the safety of linezolid when administered to patients on concomitant medications that might put them at risk from MAO inhibition. Therefore, linezolid is not recommended for use in these circumstances unless close observation and monitoring of the recipient is possible.

Potential interactions producing elevation of blood pressure: Linezolid enhanced the increases in blood pressure caused by pseudoephedrine and phenylpropanolamine hydrochloride. Co-administration of linezolid with either pseudoephedrine or phenylpropanolamine resulted in mean increases in systolic blood pressure of the order of 30-40 mmHg, compared with 11-15 mmHg increases with linezolid alone, 14-18 mmHg with either pseudoephedrine or phenylpropanolamine alone and 8-11 mmHg with placebo. It is recommended that doses of drugs with a vasopressive action, including dopaminergic agents, should be carefully titrated to achieve the desired response when co-administered with linezolid.

Potential serotonergic interactions: Post marketing experience: there has been one report of a patient experiencing serotonin syndrome-like effects while taking linezolid and dextromethorphan which resolved on discontinuation of both medications.

During clinical use of linezolid with serotonergic agents, including antidepressants such as selective serotonin reuptake inhibitors (SSRIs), cases of serotonin syndrome have been reported. Therefore, while co-administration is contraindicated, management of patients for whom treatment with linezolid and serotonergic agents is essential.

Use with tyramine-rich foods: It is only necessary to avoid ingesting excessive amounts of food and beverages with a high tyramine content (e.g. mature cheese, yeast extracts, undistilled alcoholic beverages and fermented soya bean products such as soy sauce).

Drugs metabolised by cytochrome P450: Linezolid is not detectably metabolised by the cytochrome P450 (CYP) enzyme system and it does not inhibit any of the clinically significant human CYP isoforms (1A2, 2C9, 2C19, 2D6, 2E1, 3A4). Similarly, linezolid does not induce P450 isoenzymes in rats. Therefore, no CYP450-induced drug interactions are expected with linezolid.

Rifampicin: Rifampicin decreased the linezolid Cmax and AUC by a mean 21% [90% CI, 15, 27] and a mean 32% [90% CI, 27, 37], respectively. The mechanism of this interaction and its clinical significance are unknown.

Warfarin: When warfarin was added to linezolid therapy at steady-state, there was a 10% reduction in mean maximum INR on co-administration with a 5% reduction in AUC INR. There are insufficient data from patients who have received warfarin and linezolid to assess the clinical significance, if any, of these findings.

PREGNANCY AND LACTATION

Pregnancy
There are limited data from the use of linezolid in pregnant women. However, a potential risk for human reproductive toxicity has been shown.

Linezolid should not be used during pregnancy unless clearly necessary only if the potential benefit outweighs the theoretical risk.

Lactation
Animal data suggest that linezolid is transferred into the breast milk. It is not known whether linezolid is excreted in human milk. Therefore, caution should be exercised when linezolid is administered to a nursing woman.

Fertility
It is suggested that linezolid has caused a reduction in fertility.

ADVERSE REACTIONS

Infections and infestations
Common: candidiasis, oral candidiasis, vaginal candidiasis, fungal infections
Uncommon: vaginitis
Rare: antibiotic-associated colitis including pseudomembranous colitis

Blood and the lymphatic system disorders
Common: anaemia
Uncommon: leucopenia, neutropenia, thrombocytopenia, eosinophilia
Rare: pancytopenia
Frequency not known: myelosuppression, sideroblastic anaemia

Immune system disorders
Frequency not known: anaphylaxis

Metabolism and nutrition disorders
Uncommon: hyponatraemia
Frequency not known: lactic acidosis

Psychiatric disorders
Common: insomnia

Nervous system disorders
Common: headache, taste perversion (metallic taste), dizziness
Uncommon: convulsions, hypoaesthesia, paraesthesia
Frequency not known: serotonin syndrome, peripheral neuropathy

Eye disorders
Uncommon: blurred vision
Rare: changes in visual field defect
Frequency not known: optic neuropathy, optic neuritis, loss of vision, changes in visual acuity, changes in colour vision

Ear and labyrinth disorders
Uncommon: tinnitus

Cardiac disorders
Uncommon: Arrhythmia (tachycardia)

Vascular disorders
Common: hypertension
Uncommon: transient ischaemic attacks, phlebitis, thrombophlebitis

Gastrointestinal disorders
Common: diarrhoea, nausea, vomiting, localised or general abdominal pain, constipation, dyspepsia
Uncommon: pancreatitis, gastritis, abdominal distention, dry mouth, glossitis, loose stools, stomatitis, tongue discolouration or disorder
Rare: superficial tooth discolouration

Hepato-biliary disorders
Common: abnormal liver function test; increased AST, ALT or alkaline phosphatase
Uncommon: increased total bilirubin

Skin and Subcutaneous tissue disorders
Common: pruritus, rash
Uncommon: urticaria, dermatitis, diaphoresis
Frequency not known: bullous disorders such as those described as Stevens-Johnson syndrome and toxic epidermal necrolysis, angioedema, alopecia

Renal and urinary disorders
Common: increased BUN
Uncommon: renal failure, increased creatinine, polyuria

Reproductive system and breast disorders
Uncommon: Vulvovaginal disorder

General disorders
Common: fever
Uncommon: chills, fatigue, increased thirst

Investigations
Common:
Chemistry
Increased LDH, creatine kinase, lipase, amylase or non-fasting glucose. Decreased total protein, albumin, sodium or calcium. Increased or decreased potassium or bicarbonate.

Haematology
Increased neutrophils or eosinophils. Decreased haemoglobin, haematocrit or red blood cell count. Increased or decreased platelet or white blood cell counts.
Uncommon:

Chemistry
Increased sodium or calcium. Decreased non fasting glucose. Increased or decreased chloride.

Haematology
Increased reticulocyte count. Decreased neutrophils.

The following adverse reactions to linezolid were considered to be serious in rare cases: transient ischaemic attacks and hypertension.

OVERDOSE AND TREATMENT

No specific antidote is known.
No cases of overdose have been reported.
However, supportive care is advised together with maintenance of glomerular filtration. Approximately 30% of a linezolid dose is removed during 3 hours of haemodialysis. The two primary metabolites of linezolid are also removed to some extent by haemodialysis. It is not known if linezolid is removed by peritoneal dialysis or haemoperfusion.

STORAGE CONDITIONS

Store below 30°C. Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

DOSAGE FORMS AND PACKAGING AVAILABLE

In box of 10 tablets (1 blister x 10 tablets).

PRODUCT REGISTRATION HOLDER/MANUFACTURER:

PHARMANIAGA MANUFACTURING BERHAD (198001006232)
No.11A, Jalan P/1, Kawasan Perusahaan Bangi,
43650 Bandar Baru Bangi, Selangor Darul Ehsan, Malaysia

Date of revision: 18 April 2022

PRP XXXXXXXXXXX

212mm x 277mm Font Size: 6.5pt